

SESSION 1 INTAKE PREP SHEET: ASSESSMENT + FIRST INDUCTION

TM-002 (T.M.) | Wed Oct 8, 6:00 PM PST | Zoom | **THERAPIST ONLY**

BEFORE SESSION STARTS: Confirm SB 577 + Recording Consent are SIGNED. This client has childhood trauma history — screen for contraindications before any induction work. If active psychiatric care or suicidal ideation is present, REFER to licensed MHP.

PRE-SESSION CHECKLIST

- ☐ SB 577 + Recording Consent signed?
 - ☐ Review presenting issue (below)
 - ☐ Prepare suggestibility test (36 questions)
 - ☐ Contraindication screening questions ready
 - ☐ L.O.V.E. intake structure reviewed
- ☐ Zoom audio/video tested
 - ☐ Recording software ready (start at induction if consented)
 - ☐ Notepad ready for EP scoring
 - ☐ Timer ready for induction duration
 - ☐ CSP: CCH log form ready for hours tracking

CLIENT PROFILE (FROM REGISTRATION)

FIELD	VALUE
Case ID	TM-002 (TRAUMA-001)
Age	22
Referral	HMI Community Service Program (CSP) — pro bono
Prior therapy	Traditional psychotherapy ("helped a ton" but unresolved feelings remain)
Preference	Requested male therapist for comfort discussing men's issues
EP	TBD (determine via 36-question test this session)

PRESENTING ISSUES (CLIENT'S OWN WORDS)

"I have some issues with **letting go of past traumatic experiences** and the feelings surrounding them from when I was a young kid. I've tried traditional psychotherapy, which has helped a ton, but I am still looking to resolve complicated feelings around **self-esteem, confidence, assertiveness**, and generally to **feel comfortable in my own skin**. I have issues **letting go** and **trusting people** that are close to me. It still feels like my **anxiety still runs the show** sometimes, and I want to **become more open and less hypervigilant**."

ISSUE MAP (6 PRESENTING THEMES)

THEME	CLIENT'S LANGUAGE	POSSIBLE SUBCONSCIOUS ROOT	APPROACH
1. Childhood trauma	"past traumatic experiences from when I was a young kid"	Core wound — drives all other symptoms	Inner child work, age regression (with caution), reframe
2. Self-esteem	"feel comfortable in my own skin"	Core belief: "I am not enough / not safe being me"	Core belief reframe, mirror work, self-acceptance
3. Confidence / assertiveness	"confidence, assertiveness"	Learned helplessness from childhood — voice was suppressed	Assertiveness installation, boundary anchoring
4. Trust issues	"trusting people that are close to me"	Attachment disruption — close = danger	Safe connection reframe, circle of trust visualization
5. Anxiety / hypervigilance	"anxiety still runs the show", "hypervigilant"	Threat detection system stuck ON from childhood	Dial-down anchor, nervous system recalibration
6. Letting go	"issues letting go"	Holding on = protection strategy, letting go = vulnerability	Release ritual, container technique, forgiveness work

CLINICAL NOTE: These 6 themes likely share a single root: the childhood trauma. Do NOT treat them as separate issues. Session 1 should identify the root and map the connection. The treatment arc should work from root (trauma) outward to symptoms (anxiety, trust, confidence). Psychotherapy gave cognitive insight — hypnotherapy accesses the subconscious patterns that insight alone could not resolve.

CRITICAL SCREENING (DO BEFORE ANY INDUCTION)

Ask directly. Do not skip any of these:

- Active psychiatric care?** "Are you currently seeing a psychiatrist, psychologist, or therapist?" — If yes, get name/details. Coordinate, do not conflict.
- Medications?** "Are you currently taking any medications — anti-anxiety, anti-depressant, anti-psychotic, or anything else?" — If anti-psychotic or active psychosis: REFER. If anti-anxiety/anti-depressant: proceed with awareness.
- Suicidal ideation?** "Have you ever had thoughts of harming yourself or ending your life?" — If active: STOP. Refer to 988 Suicide & Crisis Lifeline. Do not proceed with hypnosis.

4. **Trauma details?** "You mentioned experiences from childhood. Without needing to share details today, can you tell me — was this a single event or ongoing? Were you ever physically harmed?" — Scope the trauma WITHOUT forcing disclosure. Session 1 is assessment, not processing.
5. **Triggers?** "Is there anything I should be careful about — words, topics, or situations that tend to trigger strong reactions for you?" — Know before you go into trance.
6. **Panic attacks?** "Do you experience panic attacks? How frequently?" — If yes, adjust induction (avoid claustrophobic imagery, use open-space visualizations).

TIME ALLOCATION (60-90 MIN)

PART	TIME	WHAT	NOTES
1	20-25 min	Intake + screening + rapport	L.O.V.E. structure. Listen first. Do NOT rush to solutions. Let him tell his story.
2	10-15 min	Suggestibility test (36 questions)	Score EP. Determines lane for entire treatment arc.
3	10-15 min	Theory of Mind + education	Conscious/subconscious, fight-flight-freeze, how hypnosis accesses what talk therapy cannot. Frame hypnotherapy as complement to psychotherapy.
4	15-20 min	First induction + deepeners	ONLY if screening clears. Gentle approach. No age regression in Session 1. Safe space visualization only.
5	5-10 min	Count out + homework + next steps	PHS verify. Assign homework. Schedule Session 2.

INDUCTION STRATEGY (ADAPT TO EP RESULT)

If Physical (direct, literal)

- Arm-raising induction (eagle or magnetic pull)
- Direct language: "you ARE relaxing"
- Kinesthetic anchors: weight, temperature, pressure
- Challenge deepeners: hand-forehead, arm rigidity
- Reactional deepener: rapid re-entry

If Emotional (permissive, indirect)

- Eye-fixation or eye-closure induction
- Permissive language: "you may notice..."
- Emotional anchors: safety, warmth, permission
- Progressive relaxation (slow, gentle)
- Guided imagery deepener (safe place)

TRAUMA-SPECIFIC INDUCTION RULES:

- NO age regression in Session 1 — only safe-space building
- NO "go back to a time when..." — too early, no trust established
- Include permission language: "with your consent", "only as comfortable"
- Build a safe space visualization (his design, not yours) — this becomes the anchor for future trauma work
- If abreaction occurs: stay calm, deepen trance, guide to safe space, do NOT count out abruptly
- PHS wording: add "safely and comfortably" to standard formula

SESSION 1 THERAPY GOALS (ASSESSMENT ONLY)

1. **Build rapport and trust** — he requested a male therapist for comfort. Honor that. Be direct but warm.
2. **Complete screening** — medications, active care, suicidal ideation, trauma scope, triggers, panic
3. **Determine EP** — 36-question test, score Physical/Emotional split
4. **Educate** — how hypnosis works, how it complements psychotherapy, what to expect
5. **First induction experience** — demonstrate trance, build confidence in the process
6. **Establish safe space** — guided visualization of a place that feels completely safe (his design)
7. **Assess SMART goals** — baseline each of the 6 themes on 1-10 scale

SMART GOALS BASELINE (COLLECT DURING SESSION)

MEASURE	ASK CLIENT (1-10)	BASELINE
Self-esteem ("comfort in own skin")	"Right now, how comfortable do you feel being yourself?"	___/10
Assertiveness	"How easy is it for you to set boundaries or say no?"	___/10
Anxiety level	"On a typical day, how much does anxiety run the show?"	___/10
Trust in close relationships	"How much do you trust the people closest to you?"	___/10
Comfort in own skin	"How often do you feel at ease just being you?"	___/10
Letting go of past	"How much does the past still control your present?"	___/10

END OF SESSION CHECKLIST

- ☐ Screening complete — all 6 questions asked and documented
- ☐ EP determined: Physical ___% / Emotional ___%
- ☐ SMART baselines collected (6 measures)
- ☐ Safe space established (describe: _____)
- ☐ Count out: gentle, grounding (not rapid for trauma client)

- ☐ PHS verify (finger-spread)
- ☐ Homework: journaling about safe space, practice 4-7-8 breathing for grounding
- ☐ AI disclosure given
- ☐ Schedule Session 2 (1-2 weeks)
- ☐ Generate AVS: `python scripts/generate_client_docs.py data/clients/TM-002.json`
- ☐ Log CCH hours to HMI
- ☐ Update TM-002.json with EP, SOAP, session data

RED FLAGS DURING SESSION

- **Abreaction:** If client becomes visibly distressed during trance — deepen, guide to safe space, use grounding language. Do NOT count out suddenly.
- **Dissociation:** If client goes blank/unresponsive — ground with physical cues ("feel your feet on the floor, feel the chair"). Bring back slowly.
- **Disclosure of abuse:** If client discloses abuse of a minor or elder — mandatory reporting obligation. Document and follow CA reporting procedures.
- **Suicidal ideation disclosed:** Stop session. Provide 988 Suicide & Crisis Lifeline. Do not continue hypnosis. Refer to licensed MHP.
- **Active psychosis signs:** Disordered thinking, hallucinations, paranoid ideation — refer immediately. Hypnosis contraindicated.

FRAME FOR CLIENT: "You've already done significant work in psychotherapy. That gave you conscious insight. What we're going to do is different — we're going to work with the subconscious patterns that insight alone hasn't been able to shift. Think of psychotherapy as updating the user manual, and hypnotherapy as updating the firmware."